

CHRONIC PAIN

Chronic pain is pain that lasts longer than 4 weeks. A doctor should confirm the underlying cause of the pain.

Assess the patient with chronic pain at every visit

Assess	Note
Site and duration of pain	Ask where the pain is and when the pain started. Does pain radiate anywhere?
Type of pain	- Does patient have cancer pain or non-cancer pain? - If non-cancer pain, decide if patient has tissue pain, nerve pain or central pain: - If arthritis, joint pain, lower back/neck pain or chronic lung problem, tissue pain likely. - If previous shingles, trigeminal neuralgia, peripheral neuropathy or diabetic neuropathy, nerve pain likely - If fibromyalgia or irritable bowel syndrome, central pain likely.
Severity of pain	- Does pain limit activity or affect sleep, activities of daily living, mood or social/work functioning? - Ask patient to grade pain on a scale from 0 - 10, with 0 being no pain and 10 being the worst pain: classify pain as mild (1-3), moderate (4-7) or severe (8-10).
Relieving and aggravating factors	What makes the pain better or worse? What has patient been using to help pain? Does pain medication help? Use this to help patient decide which pain strategies will help the most.
Mental health	Ask how patient is coping and what support and/or spiritual care is needed. If low mood, stress or anxiety ➤ 86.
Alcohol/drug use	In the past year, has patient: 1) drunk ≥ 4 drinks ¹ /session, 2) used illegal drugs or 3) misused prescription or over-the-counter medications? If yes to any ➤ 142.
Chronic conditions	Ensure patient is receiving appropriate care for all his/her other chronic conditions. Manage as on routine care page/s.
Palliative care	If patient has a life-limiting illness, also consider giving palliative care ➤ 170.
Examine area of pain	If skin problem ➤ 67. If joint problem ➤ 62. If back, neck, leg, foot, arm or hand problem, manage as on symptom page.

Advise the patient with chronic pain

- Ask what patient thinks is causing pain and what impact it is having on his/her life and family. Address patient's and family's concerns about pain. For tips on communicating effectively ➤ 176.
- Educate that non-cancer chronic pain can be because pain signals in the brain stop working normally and get stuck in the 'on' position, even when the cause has resolved. It does not always mean a disease or cancer. Tests cannot always show the reason for the pain and often are not needed.
- Help the patient, along with his/her family, to choose strategies to get help and cope with chronic pain:

Get enough sleep

If patient has difficulty sleeping, give advice ➤ 87.



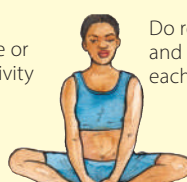
Encourage patient to take time to relax:



Spend time with supportive friends or family.



Find a creative or fun activity to do.



Do relaxing breathing and stretching exercises each day.



Manage activities

Advise to stagger tasks and to avoid trying to fit too many activities into one day.



Get active

Aim for at least 30 minutes of moderate exercise most days. Start with 10 minutes/day and increase steadily.



Access support

Link patient with helpline or support group ➤ 178.

Limit alcohol and avoid drugs

- Limit alcohol to ≤ 2 drinks/day and avoid alcohol on at least 2 days/week.
- Avoid drugs and unnecessary pain medication.



Treat the patient with chronic pain

- Manage pain together with patient and interdisciplinary team: refer if needed to physiotherapist, occupational therapist, spiritual counsellor/psychologist, social worker and community health worker.
- Together with the patient, set realistic pain management goals: to be as pain-free as possible (this might not be completely pain-free) so that s/he can do the things s/he wants and enjoys doing.
- Treat the patient's chronic pain on relevant symptom or routine care page. If appropriate, suggest patient tries movement exercise and heat/cold packs as these may help ease the pain.

¹One drink is 1 tot of spirits, or 1 small glass (125mL) of wine or 1 can/bottle (330mL) of beer.